



EVERGREEN COMMUNITY CARE INC.

PROGRAM COORDINATOR MANUAL

All Tasks Aligned to MCFD Standards for Staffed Children's Residential Services (SHSS)

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Applies To	Program Coordinators in all Evergreen Community Care Residential Programs
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Related Documents	Residential Care Program Manual (ECC-RC-POL-2026-003); HR Policy Binder (ECC-HR-POL-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); QA Program Binder (ECC-QA-2026-001)
Legislative Framework	CFCSA; CCALA; Residential Care Regulation (RCR, amended April 2026); DRIPA; Workers Compensation Act; BC Human Rights Code; FOIPPA



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PART 1 — ROLE, AUTHORITY, AND ACCOUNTABILITY

1. ROLE OF THE PROGRAM COORDINATOR

Authority: MCFD Standards G.4, H.1; HR Policy Binder s.2; Residential Care Regulation s.66–80

1.1 Position Summary

The Program Coordinator is the operational hub of Evergreen Community Care's residential programs. Reporting to the Program Manager and supervising direct-care staff, the Program Coordinator is responsible for the day-to-day implementation of MCFD Standards A through H across the assigned program. The Program Coordinator does not set policy — they implement it, monitor compliance with it, and escalate gaps in it.

1.2 Reporting Structure

Reports To	Program Manager
Supervises	Youth Care Workers; Residential Support Workers; Relief Staff
Works Closely With	MCFD Social Workers; Health Authority; School Teams; DAA / Indigenous Community Partners; Other Program Coordinators
Escalates To	Program Manager for any matter beyond the PC's authority or resources
On-Call Role	Designated on-call supervisor on a rotating schedule; available to all on-duty staff 24/7 during on-call periods

1.3 Authority Limits

⚠ The Program Coordinator DOES NOT have authority to: make verbal employment offers; commit the organization to contractor services; authorize expenditures above the delegated limit (see Finance Manual); accept new placements without Program Manager confirmation; modify the ISP or care plan without MCFD social worker approval. All such actions require Program Manager or Executive Director sign-off.

1.4 Core Accountability Areas

Accountability Area	Primary Standard
Staff supervision and performance	Standard G.4



Staff recruitment screening and records	Standard G.1
Staffing ratios and scheduling	Standard G.2
Training compliance	Standard G.3
Youth rights and complaint response	Standard B
Personal safety and incident reporting	Standard C
Youth care quality — daily environment	Standard D
Service planning and ISP coordination	Standard E
Physical facility and safety	Standard F
Program description and purpose compliance	Standard A
Administration, records, and QA	Standard H
OHS implementation at facility level	Workers Compensation Act / OHS Regulation



PART 2 — MCFD STANDARD A: PROGRAM PURPOSE AND DESCRIPTION

2. STANDARD A — PURPOSE AND FUNCTION OF THE PROGRAM

MCFD SHSS Standard A.1; Residential Care Regulation s.7; CCALA

2.1 PC Responsibilities Under Standard A

The Program Coordinator is responsible for ensuring that the program operates in accordance with its written Program Description and that all staff understand and apply the program's stated purpose, philosophy, and practices.

Task	Standard Requirement	PC Action
Maintain written Program Description	A.1 — written program description required	Review the Program Description at each ISP review cycle; report any practice gaps to Program Manager
Distribute Program Description to new youth	A.1 — provided to each child/youth at admission	Ensure admitting staff provide the Program Description (or accessible summary) at every admission; verify via Admission Checklist
Distribute to new staff	A.1 — provided to all staff	Provide Program Description at orientation; obtain signed acknowledgement; file in personnel record
Review annually	A.1 — annual review required	Participate in annual Program Description review in July; flag any outdated content to Program Manager
Maintain staff name/title list	H.1 — current list maintained and shared with MCFD	Keep the staff directory current; update within 5 business days of any staffing change; provide to MCFD on request
Operate within licensed capacity	CCALA / Licensing	Never admit a youth that would exceed licensed capacity; confirm capacity with Program Manager before any admission





PART 3 — MCFD STANDARD B: RIGHTS, COMPLAINTS, AND PRIVACY

3. STANDARD B — RIGHTS, COMPLAINT RESOLUTION, AND PRIVACY

MCFD SHSS Standards B.1–B.5; CFCSA s.70; Child in Care Regulation; FOIPPA

3.1 Youth Rights — PC Responsibilities

Task	Standard / Authority	Timeline
Ensure rights information is provided to every youth at admission	B.1 — each child/youth informed of rights	Same day as admission; verify via Admission Checklist
Ensure rights posting in common area is current	B.1 — posted in accessible location	Check monthly; replace immediately if outdated or damaged
Ensure emergency contacts are posted at every telephone	CFCSA s.70; B.1	Confirm at every facility inspection; update within 24 hours of any contact change
Ensure youth know how to contact the Advocate (1-800-476-3933) privately	B.1 — assistance accessing RCY	Verify at each youth's first week check-in; facilitate private phone access without restriction at any time
Never restrict access to the Advocate, Ombudsperson, or police	CFCSA s.70 — absolute right	Ongoing; any restriction reported immediately to Program Manager as a rights violation and CI

3.2 Complaint Resolution — PC Responsibilities

The Program Coordinator is the first level of escalation for formal complaints. The process:

Step	PC Action	Timeline
Complaint received (by any staff member)	Acknowledge; document in shift log and on Complaint Form; notify Program Manager	Same shift as received



Acknowledgement to complainant	Confirm receipt in person or in writing	Within 24 hours
Investigate	Gather facts; review documentation; speak with involved parties	Within 5 business days
Written response	Provide written response with findings and any corrective actions	Within 10 business days of receipt
Escalation	If complainant is not satisfied, refer to Program Manager	Within 5 business days of escalation request
External referral	Facilitate contact with MCFD, RCY, Ombudsperson, or licensing officer — without restriction	Immediately, at any stage, at the complainant's choice

⚠ No youth or family member is ever discouraged, pressured, or deterred from making a complaint or contacting external authorities. Any staff member who interferes with this right is subject to immediate disciplinary action. If you observe or receive a report of this, treat it as a Critical Incident.

3.3 Privacy and FOIPPA — PC Responsibilities

- Ensure all ISRs are stored in locked files; electronic records in access-controlled systems
- Authorize access to ISRs only for staff with a current, legitimate care need
- Refer all requests for records from police, courts, media, or family members to the Program Manager immediately — never release records without legal authority
- Ensure no staff member discusses youth information on social media or with unauthorized persons
- Respond to any suspected privacy breach immediately: contain; notify Program Manager; document; follow FOIPPA breach response procedure
- Ensure no youth is photographed or identified publicly without written MCFD authorization



PART 4 — MCFD STANDARD C: PERSONAL SAFETY (SHSS STANDARD C)

4. STANDARD C — PERSONAL SAFETY OF CHILDREN AND YOUTH

MCFD SHSS Standards C.1–C.9; Residential Care Regulation s.57–70; OHS Regulation

4.1 Environmental Safety — PC Responsibilities

Task	Standard / Authority	Frequency
Conduct or delegate monthly facility safety inspection using Appendix C checklist	C.1; F.4; OHS Reg. s.3.5	Monthly — first week of each month
Verify hot water temperature at all taps (maximum 49°C per RCR April 2026)	C.1; RCR s.36 (April 2026 amendment)	At every admission and monthly inspection
Verify all exterior doors and windows are secured per facility protocol	C.1	At start of every shift; verify at facility inspection
Confirm medication cabinet locked; controlled substance count reconciled	C.1; RCR s.46	At every shift change — both outgoing and incoming staff joint count
Confirm chemical storage locked; SDS binder current	C.1; OHS Reg. Part 5 (WHMIS)	Monthly inspection; update SDS within 30 days of any product change
Confirm fire safety equipment operational (detectors, extinguishers, exits, signs)	F.4; BC Fire Code; OHS Reg.	Monthly; record on inspection form
Conduct environmental risk assessment when youth with specific risk profile is	C.1; Safety Planning requirement	Within 24 hours of admission; document; implement modifications before or within 24 hours



admitted		
Review and update Environmental Risk Assessment annually	C.1; MCFD Standards F.1	Annually in July; also following any critical incident revealing an environmental hazard

4.2 Critical Incident — PC Responsibilities

⚠ The Program Coordinator is the first management response to all Critical Incidents during shifts when the Program Manager is not on site. When in doubt about whether something is a CI, treat it as one. Err toward reporting.

Incident Category	PC Immediate Action	Timeline
Category 1 (death, hospitalization, abuse allegation, suicidal crisis, missing youth, restraint use, police contact, school suspension)	Ensure 911 called if needed; receive staff notification; notify Program Manager; ensure MCFD social worker or PCS (1-800-663-9122) is notified; ensure CI Report is started	Immediate notification; written CI Report within 2 hours for restraint/violence; within 24 hours for others
Category 2 (marked behaviour change, medication error with medical attention, property damage over \$500, unauthorized overnight)	Notify Program Manager; ensure CI Report completed	By end of next business day
Any Category 1 when Program Manager unavailable	Follow all Category 1 steps; notify Executive Director if Program Manager unreachable after two attempts	Same timeline; never delay because of supervisor unavailability

4.3 Post-CI Review — PC Responsibilities

- Within 5 business days of every Category 1 CI: review the CI Report; identify contributing factors; document corrective actions with named owners and target dates; update ISP and BSP; present findings to the Program Manager
- Track all corrective action items to completion; bring overdue items to the next supervision meeting
- Ensure all involved staff are offered EFAP access before the end of the shift following any significant incident



- Notify the JHSC within 24 hours of any CI with worker safety implications

4.4 Physical Restraint — PC Oversight Responsibilities

- Confirm all direct-care staff hold current NVCI certification before authorizing solo shifts — check Appendix E monthly
- Respond to every reported use of physical restraint within the shift it occurred: verify medical wellness check done; verify youth debrief planned; verify MCFD notified; sign the CI Report
- Track all restraint incidents monthly; report trend of 3+ restraints involving the same youth in 30 days to the Program Manager as a threshold event requiring urgent ISP review
- Review prohibited practices list with the team at minimum annually; address any observed or reported prohibited practice as a Category 1 CI

4.5 Missing Youth — PC Responsibilities

Step	PC Action	Timeline
Staff notifies PC of missing youth	Confirm Phase 1 (prudent parent steps) are underway; begin chronological log	Immediately on notification
Youth confirmed missing (Phase 1 not resolved in 30 min)	Notify Program Manager; ensure MCFD social worker or PCS notified; confirm 911 / RCMP called per protocol	Immediately on Phase 2 activation
While youth is missing	Coordinate ongoing search efforts; maintain chronological log; update Program Manager every 30 minutes	Continuously while missing
Youth returns	Direct welfare check; ensure supervisor and MCFD notified; arrange debrief within 24 hours	Immediately on return
ISP review post-return	Ensure ISP and safety plan updated within 5 business days	Within 5 business days



PART 5 — MCFD STANDARD D: QUALITY OF SERVICE EXPERIENCES

5. STANDARD D — QUALITY OF SERVICE EXPERIENCES

MCFD SHSS Standards D.1–D.8; Child in Care Regulation; Residential Care Regulation s.7–50

5.1 Daily Care Environment — PC Oversight

The Program Coordinator is responsible for ensuring that the daily environment of the program meets MCFD Standards D.1 through D.8 at all times. This is achieved through: regular observation of shifts; review of shift logs; team meetings; and the monthly QA review.

Standard D Element	PC Monitoring Action	Frequency
D.1 — Service environment: cultural safety, gender affirmation, dignity	Observe staff interactions; review shift logs for language and tone; address any observed rights violation immediately	Each shift visited; monthly shift log review
D.1.13(b) — Gender-affirming care (amended April 2024)	Confirm all staff use correct names and pronouns; respond to any misgendering as a Code of Ethics violation	Ongoing; any incident treated as CI
D.2 — Family involvement in care	Review contact logs monthly; ensure Family Contact Plans are active; address any unexplained barriers to authorized contact	Monthly
D.3 — Maintaining relationships	Confirm family contact occurring per plan; for Indigenous youth — confirm Nation and DAA connections are facilitated	Monthly QA review
D.4 — Community inclusion	Confirm community outings documented per ISP; review outing safety plans (Appendix K)	Monthly
D.5 — Access to services and community	Confirm no youth is being restricted from authorized community access without documented ISP-based reason	Monthly QA review
D.6 — Growth, development, education	Confirm school attendance tracked; confirm educational goals in ISP; follow up on school absences	Weekly review of shift logs; monthly QA
D.7 — Behaviour management: positive practices	Review any restraint or consequence decisions; provide immediate correction for any prohibited practice; address in next team meeting	Within 24 hours of any reported incident



only		
D.8 — Decision-making and youth voice	Confirm ISP records genuine youth input; ensure youth feedback mechanisms are accessible	Monthly file audit sample; bi-annual formal review

5.2 Beverage and Water Temperature Safety (RCR April 2026 Amendment)

- Confirm at every monthly inspection that hot water at all taps does not exceed 49°C (thermostatic mixing valves operational)
- Ensure staff are trained to check temperature of all hot beverages, formula, and heated foods provided to youth before serving
- Document temperature check procedure in orientation for all new staff

5.3 Allowances and Financial Management

- Confirm youth allowances are paid consistently on the scheduled day; track in the program financial log
- Ensure allowances are never withheld as a consequence — this is a violation of CFCSA s.70
- Submit allowance expenditure records to Program Manager monthly per Finance Manual requirements
- Maintain youth trust fund records where applicable; account for all transactions in writing



PART 6 — MCFD STANDARD E: CHILD AND YOUTH SERVICE PLANNING

6. STANDARD E — CHILD AND YOUTH SERVICE PLANNING

MCFD SHSS Standards E.1–E.7; CFCSA s.70(1)(b); MCFD Child Welfare Policy Chapter 4

6.1 Admission — PC Responsibilities (Standard E.1)

Task	Standard / Authority	Timeline
Review referral documentation and assess program fit	E.1 — admission criteria	Before confirming admission; 48 hours maximum for emergency placements
Confirm licensed capacity has room for the placement	CCALA; licensing conditions	Before admission confirmed
Complete Admission Checklist (Appendix B)	E.1; Standard C.1	Day 1 of admission; all items checked before youth arrives where possible
Confirm CRC is on file for all staff who will work with the new youth	G.1; Criminal Records Review Act	Before admission; any lapse reported to Program Manager immediately
Conduct environmental risk assessment for this youth's specific risk profile	C.1; Safety Planning	Within 24 hours of admission
Initiate preliminary ISP within 48 hours	E.5; Standard C.1	Within 48 hours
Schedule full ISP meeting within 14 days	E.5	Within 48 hours of admission; meeting within 14 days
Confirm Indigenous status; schedule Cultural Plan meeting within 30 days	CFCSA s.70; DRIPA	Day 1 confirmation; Cultural Plan meeting within 30 days
Ensure orientation completed with the	E.1; Standard B.1	Within 48 hours of arrival



youth (Appendix B)

6.2 Individual Service Plan (ISP) — PC Responsibilities (Standards E.4–E.5)

Task	Standard	Timeline
Facilitate ISP development meeting	E.5 — collaborative development with youth	Within 14 days of admission
Ensure youth's voice is documented genuinely in the ISP	E.5; CFCSA s.70(1)(b)	At every ISP development and review meeting
Coordinate care team participation (MCFD SW, family, school, health)	E.5 — integrated planning	For all ISP meetings
Distribute ISP to all direct-care staff; confirm read and understood	E.5; G.4	Immediately after MCFD approval; confirm by team meeting
Review BSP with all staff at shift handover during high-risk periods	C.3; E.5	Ongoing; especially following any incident
Schedule quarterly ISP reviews	E.5 — minimum 90-day review cycle	Track in the program calendar; never miss without MCFD approval
Trigger emergency ISP review within 5 business days of any CI	E.5; C.2	Within 5 business days of any Category 1 CI
Ensure Cultural Plan developed within 30 days for every Indigenous youth	CFCSA s.71; DRIPA	Within 30 days of admission; track on QA calendar
Monitor ISP goal progress through shift log review	E.5; H.1	Weekly shift log review; monthly QA tool
Coordinate	E.6 — transition planning	60 days before anticipated



transition planning beginning 60 days before discharge		transition; update monthly
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6.3 School and Education — PC Responsibilities (Standard D.6)

- Confirm every school-aged youth is enrolled in an appropriate educational program within 5 school days of admission
- Establish contact with the school on admission day; introduce yourself and the youth's primary staff contact
- Review school attendance weekly from shift logs; follow up on any unexplained absence the same day it occurs
- Report school suspensions to the MCFD social worker as a Category 1 Critical Incident on the same day
- Attend school meetings (parent-teacher, IEP reviews, case conferences) on behalf of Evergreen Community Care
- Escalate to Program Manager any situation where a youth's school access is being denied or significantly disrupted

6.4 Health Care — PC Responsibilities (Standard D.6)

- Confirm every youth is established with a family physician or NP within 30 days of admission; track on the admission checklist
- Maintain a health care appointment log for each youth; ensure all appointments are scheduled, attended, and documented
- Confirm 911 is called immediately for any medical emergency; ensure staff know MCFD SW must be notified within 1 hour
- Review medication records weekly: confirm MAR is current, complete, and signed; flag any errors or gaps immediately
- Ensure controlled substance counts are completed and reconciled at every shift change; investigate any discrepancy immediately



PART 7 — MCFD STANDARD F: SHELTER AND EQUIPMENT

7. STANDARD F — SHELTER AND EQUIPMENT

MCFD SHSS Standards F.1–F.4; Residential Care Regulation s.22–56; BC Building Code; BC Fire Code

7.1 Physical Facility — PC Responsibilities

Task	Standard / Authority	Frequency
Conduct complete monthly facility safety inspection (Appendix C)	F.4; OHS Reg. s.3.5	Monthly — first week
Confirm current facility licence is posted prominently	CCALA / Licensing	Monthly inspection; update immediately when renewed
Confirm hot water temperature at all taps $\leq 49^{\circ}\text{C}$	F.1; RCR April 2026	Monthly inspection and at each new admission
Confirm smoke and CO detectors in all sleeping areas and common spaces	F.4; BC Fire Code	Monthly test and log; replace batteries as needed
Confirm fire extinguishers charged, tagged, and accessible	F.4; BC Fire Code	Monthly inspection; annual inspection by certified contractor
Confirm emergency lighting operational (test monthly)	F.4; OHS Reg.	Monthly; document in inspection record
Confirm evacuation plan posted on every floor; assembly point identified	F.4; BC Fire Code	Monthly inspection; update plan whenever layout changes
Conduct quarterly fire evacuation drills (announced) and annual unannounced night drill	F.4; BC Fire Code; OHS Reg.	Quarterly (announced); annually (unannounced night/weekend)
Document all drills	F.4; OHS Reg.	Same day as drill; retain for



including time, participants, issues, and corrective actions		minimum 5 years
Report and track all facility deficiencies	F.1; H.1	Immediate report of any safety deficiency; track correction to completion
Confirm outdoor areas safe and free of hazards before youth use	F.1; C.1	Daily (shift lead); monthly formal inspection
Confirm vehicles roadworthy, insured, and first aid kit stocked	H.1; Motor Vehicle Act	Pre-trip inspection daily; monthly formal check

7.2 Maintenance and Deficiency Tracking

- Log all facility deficiencies on the inspection form with a priority level (Priority 1: immediate; Priority 2: within 48 hours; Priority 3: within 14 days)
- Track all deficiencies to confirmed resolution; no deficiency may be left without an assigned owner and target date
- Priority 1 deficiencies (imminent danger): stop work in the affected area; notify Program Manager immediately; notify WorkSafeBC if required by OHS Reg. s.3.2
- Report any deficiency that cannot be corrected within the target date to the Program Manager with an explanation and interim controls
- Maintain the deficiency log as part of the QA record; present open items at each monthly team meeting



PART 8 — MCFD STANDARD G: HUMAN RESOURCES

8. STANDARD G — HUMAN RESOURCES

MCFD SHSS Standards G.1–G.4; Residential Care Regulation s.66–80; Criminal Records Review Act; Employment Standards Act; BC Human Rights Code

8.1 Recruitment and Screening — PC Responsibilities (Standard G.1)

Task	Standard / Authority	Requirement
Participate in interviews for new direct-care staff	G.1 — personal interview required	For every candidate for the PC's team; use standardized interview questions
Confirm CRC Vulnerable Sector clearance received before any work with youth	G.1; Criminal Records Review Act	Absolute requirement; NO exceptions; PC confirms receipt in writing before first shift
Review reference checks (minimum 2 professional references)	G.1	Review all reference summaries before hiring decision; document concerns raised
Verify educational and professional qualifications	G.1 — verification of qualifications	Obtain copies of diplomas, certifications; retain in personnel file
Ensure signed offer letter on file before first shift	G.1; HR Policy s.3.2	Confirm with HR before scheduling new staff
Complete all orientation sign-offs and personnel file requirements at hire	G.1 — required personnel records	See Standard G.1 Personnel Record Requirements — complete all items at hire
Ensure youth input on desired staff qualities is documented	G.1 — youth input	Obtain and document at each ISP review; present to HR for consideration in hiring

8.2 Standard G.1 — Required Personnel File Contents



The Program Coordinator confirms that every personnel file contains the following for each direct-care staff member:

Required Item	When to Verify
Criminal Record Check (Vulnerable Sector) — current	At hire; renew every 5 years; PC confirms currency monthly
Personal and employment references — minimum 2 documented	Before hire; on file
Application / résumé and interview record	On file
Copy of offer letter and signed acceptance	Before first shift
Written acknowledgement of orientation (signed)	Day 1 of employment; filed within 24 hours
Copies of all required certifications (First Aid, NVCI, driver's licence if applicable)	On file; expiry tracked in Training Matrix (Appendix E)
Written performance evaluations (30-day, 90-day, annual)	Completed per schedule; signed by PC and employee
Documentation of any disciplinary action	Filed by PC after each progressive discipline step
Signed acknowledgement of all required training	Filed by PC as each training module is completed
Dates of commencement and any termination	On file

8.3 Staffing Ratios and Scheduling — PC Responsibilities (Standard G.2)

Task	Standard / Authority	Requirement
Maintain staffing ratios at or above licensed minimums at all times	G.2; RCR s.66	Never schedule below minimum — if a gap appears, escalate to Program Manager immediately for emergency coverage



Publish schedule minimum 2 weeks in advance	G.2; HR Policy; Fatigue Management	Posted by end of each pay period for the following 2 weeks
Confirm minimum 8-hour break between shifts for each staff member	G.2; OHS Reg. (fatigue)	At schedule creation; no exceptions without mutual written agreement and supervisor confirmation
Maintain an emergency on-call staff list	G.2 — written plan for additional emergency staff	List of casual/relief staff available for emergency coverage; reviewed quarterly
Deploy staff to reflect the age, gender, and cultural background of youth served	G.2 — complement considers youth needs	At scheduling; flag any gaps in cultural match to Program Manager
Confirm enhanced staffing for youth with active high-risk safety plans	G.2; C.1; C.3	At scheduling; match to ISP requirements; document any gap
When below-minimum ratio occurs: notify Program Manager immediately	G.2; C.1	Immediately; PC documents steps taken and outcome in the CI/shift log

8.4 Training and Staff Development — PC Responsibilities (Standard G.3)

The Program Coordinator is the front-line manager of training compliance. Monthly monitoring is mandatory.

Task	Standard / Authority	Frequency
Update the Staff Training Compliance Matrix (Appendix E)	G.3 — training records maintained	Monthly; by the 5th of each month
Remove staff from duties requiring lapsed training until compliance restored	G.3; HR Policy	Immediately on identifying any lapse; document the decision
Schedule all	G.3 — specific deadlines by training type	Ongoing; book ahead — do



required training within deadlines listed in Part 11		not wait for deadline to approach
Provide written orientation information to all new staff covering all G.3 required topics	G.3 — written orientation at hire	Day 1; signed acknowledgement by end of Week 1
Obtain signed training acknowledgements from all staff	G.3 — signed acknowledgement required	After every completed training; filed in personnel record
Present training compliance data to Program Manager monthly	G.3; H.1 — records for MCFD	Monthly; include upcoming expirations in next 60 days
Facilitate youth input on training priorities	G.3; D.8	At each annual review; document and present to Program Manager
Submit \$500 professional development requests per eligible staff annually	HR Policy s.5.3	By March 31 each year for budget planning

8.5 Staff Support and Supervision — PC Responsibilities (Standard G.4)

Task	Standard / Authority	Frequency
Provide individual supervision to all direct-care staff	G.4 — regular supervision and support	Monthly for all staff; bi-weekly for new staff (first 90 days) and staff on PIPs
Complete supervision using the required format (Appendix G)	G.4 — supervision content requirements	Every session; document and retain in personnel file
Address gender-affirming care practice in supervision	G.4.3(c) — amended April 2024	Every supervision session; not optional
Conduct 30-day,	G.4; HR Policy s.4.3	Per schedule; written



90-day, and 180-day probationary reviews		documentation; both parties sign
Conduct annual written performance evaluations	G.4 — regular written evaluations	Annually; coordinate timing with Program Manager
Issue Performance Improvement Plans when performance is below standard	G.4; HR Policy s.13	Developed with Program Manager and HR; reviewed at scheduled intervals
Apply progressive discipline per HR Policy s.14	G.4 — disciplinary procedures	Consistently; never without documentation; always with Program Manager concurrence
Facilitate team meetings (minimum monthly)	G.4 — communication mechanisms	Monthly; agenda includes: case discussion, CI review, training update, team wellbeing
Facilitate shift handover meetings (minimum 15 minutes overlap)	G.4 — shift change meetings	Every shift change; ensure handover quality via random audits
Promote a harassment-free workplace	G.4 — workplace free from harassment	Ongoing; respond to any harassment report within 24 hours
Monitor for secondary traumatic stress in staff; facilitate EFAP referrals	G.4; OHS best practice	In every supervision session; after every CI
Ensure all staff have access to EFAP information from Day 1	G.4; HR Policy s.9.7	At orientation; re-post quarterly in staff area



PART 9 — MCFD STANDARD H: ORGANIZATION, MANAGEMENT, AND QUALITY ASSURANCE

9. STANDARD H — ORGANIZATION, MANAGEMENT, AND QUALITY ASSURANCE

MCFD SHSS Standards H.1–H.2; QA Program Binder (ECC-QA-2026-001)

9.1 Administration and Records — PC Responsibilities (Standard H.1)

Task	Standard / Authority	Frequency
Maintain all ISRs in a secure, retrievable form per MCFD requirements	H.1 — records stored and accessible to MCFD	Ongoing; confirm security at monthly inspection
Spot-review shift logs weekly for documentation quality (accuracy, objectivity, completeness)	H.1 — documentation standards	Weekly; minimum 3 logs per youth reviewed
Conduct bi-annual ISR file audit using QA Tool D	H.1; MCFD Standard E.7	Bi-annually (January and July)
Ensure all staff sign policies and policy updates within 30 days of any update	H.1 — policy compliance	Within 30 days of each policy update; track and confirm in personnel records
Maintain current organizational chart and staff list; update within 5 business days of staffing change	H.1 — current chart shared with MCFD	Within 5 business days of any change; provide to MCFD on request
Maintain vehicle records and inspection logs	H.1; Motor Vehicle Act	Monthly vehicle inspection log; record all maintenance
Comply with all financial management requirements for the program budget	H.1 — accounting system	Per Finance Manual; monthly petty cash reconciliation; submit receipts within 5 business days



9.2 Quality Assurance — PC Responsibilities (Standard H.2 and QA Binder)

The Program Coordinator is the primary QA implementer at the program level. The Program Manager reviews and analyzes; the PC collects, monitors, and escalates.

QA Activity	PC Responsibility	Frequency
Monthly facility safety inspection (Tool B)	Conduct and document; track all corrective actions to completion	Monthly — first week
Monthly training compliance review	Update Appendix E; identify and address lapses; report to Program Manager	Monthly — by the 5th
Monthly shift log quality review	Review minimum 3 shift logs per youth; provide feedback in supervision	Monthly
Monthly incident log review	Review all CIs from the month; identify any emerging patterns; brief Program Manager	Monthly — end of month
Monthly QA Review Tool (Appendix D)	Complete the monthly QA dashboard; submit to Program Manager	Monthly — by the 10th
Bi-annual youth and family feedback	Administer Tool F; compile and submit results to Program Manager	January and July
Annual MCFD Standards self-assessment (Tool A)	Participate in and contribute data to the annual self-assessment	Annually in July
Annual program review	Compile program-level data for the annual review; present at the all-staff meeting	Annually in July
Corrective action tracking	Maintain the corrective action log; confirm all actions have named owners and target dates; report overdue items	Ongoing; review weekly

9.3 Reporting to Program Manager

The Program Coordinator provides the following regular reports to the Program Manager:

Report	Frequency
Verbal shift update (on-call) — any significant event	As events occur



during the shift	
Monthly QA Review Tool (Appendix D)	By the 10th of each month
Training Compliance Matrix update (Appendix E)	By the 5th of each month
Facility Inspection Report (Appendix C)	Within 3 business days of the inspection
CI Post-Review with corrective actions	Within 5 business days of every Category 1 CI
Supervision summary (Appendix G)	Filed in personnel records; summary to PM monthly
Staffing gaps or near-misses	Immediately on occurrence; written follow-up within 24 hours
Any MCFD, licensing, or WorkSafeBC inquiry or order	Immediately on receipt



PART 10 — OHS RESPONSIBILITIES BY STANDARD

10. OHS PROGRAM — PROGRAM COORDINATOR RESPONSIBILITIES

Workers Compensation Act; OHS Regulation Part 4; OHS Program Binder (ECC-OHS-POL-2026-002)

10.1 OHS Implementation Responsibilities (WCA s.115)

The Program Coordinator implements the OHS Program at the worksite level on behalf of the Program Manager. The specific OHS responsibilities of the PC are:

OHS Responsibility	Authority	Frequency
Conduct monthly workplace inspections using the OHS Inspection Checklist (Appendix C)	OHS Reg. s.3.5; WCA s.125	Monthly — first week; document deficiencies; track corrective actions
Ensure every new worker receives complete OHS orientation before first unsupervised shift	OHS Reg. s.3.22–3.23	At hire — before first solo shift; verify with signed Appendix B checklist
Investigate every incident and near-miss; complete Incident Investigation Report	OHS Reg. s.3.4	Within 24 hours; forward to Program Manager and JHSC
Submit WorkSafeBC employer reports (Form 7) within 3 days of any work-related injury requiring medical attention	WCA s.150	Within 3 business days of learning of any claimable injury; never delay
Notify WorkSafeBC immediately by phone for serious incidents (fracture, amputation,	WCA s.172	Immediately by phone: 1-888-621-7233



hospitalization, unconsciousness)		
Ensure new worker OHS orientation checklist (Appendix B) completed and signed	OHS Reg. s.3.22	At each new hire; Day 1
Conduct working-alone risk assessment for solo overnight shifts; maintain check-in log	OHS Reg. s.4.21–4.24	Before any solo overnight shift; check-in log reviewed monthly
Conduct annual workplace violence risk assessment for the facility	OHS Reg. s.4.27–4.31	Annually in July; also following any serious violent incident
Maintain the WHMIS / SDS binder — current for all chemicals in use	OHS Reg. Part 5	Update within 30 days of any product change; confirm currency at monthly inspection
Maintain the OHS bulletin board: WorkSafeBC poster current; JHSC contacts posted; emergency contacts posted	OHS Reg. / WCA s.186	Check at monthly inspection; update within 24 hours of any change
Attend JHSC meetings as the facility management representative where applicable	WCA s.125–135	Monthly; contribute facility-specific data to JHSC agenda
Ensure all PPE is available, accessible, and staff are trained in its use	OHS Reg. Part 8	Confirm at monthly inspection; restock immediately when low
Ensure controlled substance count discrepancies reported and investigated	OHS Reg. s.4.20; RCR	Immediately on identification; investigate same shift



10.2 Return-to-Work — PC Responsibilities

- Identify modified duty options for injured staff at your program level within 48 hours of any work-related injury
- Maintain regular supportive contact with staff on WorkSafeBC leave — at least every 2 weeks; not to pressure return but to maintain connection
- Ensure WorkSafeBC Form 7 is submitted within 3 business days of learning of any work-related injury requiring medical treatment
- Coordinate with Program Manager and HR on return-to-work plans; implement modified duty within the limits of the plan
- Document all RTW steps and communications in the worker's personnel file



PART 11 — MANDATORY TRAINING REQUIREMENTS — COMPLETE MATRIX

11. MANDATORY TRAINING — ALL STAFF BY MCFD STANDARD G.3

MCFD Standard G.3; OHS Regulation s.3.22–3.24; HR Policy Binder s.5; Residential Care Regulation

The following matrix governs all mandatory training at Evergreen Community Care. The PC is responsible for ensuring all direct-care staff complete each training within the required timeline. Staff with overdue mandatory training are removed from duties requiring that training until compliance is restored.

11.1 Complete Mandatory Training Matrix — All Direct-Care Staff

TRAINING	MCFD / AUTHORITY	INITIAL DEADLINE	RENEWAL
OHS Program orientation + Program Manual review	OHS Reg. s.3.22; MCFD Standard G.3	Before first independent shift — Day 1	At each annual update; document re-acknowledgement
Youth rights (CFCSA s.70) — rights of children in care	MCFD Standard G.3; CFCSA s.70	Before first solo shift — within 7 days	Every 2 years; annual refresher discussion in team meeting
Mandatory reporting duty (CFCSA s.14)	MCFD Standard G.3; CFCSA s.14	Before first solo shift — within 30 days	Every 2 years; online module mcf.gov.bc.ca
Privacy and FOIPPA — confidentiality obligations	MCFD Standard G.3; FOIPPA	Within 30 days	Annual refresher; oipc.bc.ca module
WHMIS 2015 — hazardous products in the workplace	OHS Reg. Part 5; MCFD G.3	Before first shift	Every 3 years; update immediately when new products introduced
Abuse recognition and prevention	MCFD Standard G.3	Within 30 days	Annual refresher
Missing Child/Youth Protocol (Appendix 1 per MCFD)	MCFD Standard G.3; MCFD Missing Protocol May 2025	Within 30 days	Annual refresher; update if protocol revised
Workplace Violence Prevention	OHS Reg. s.4.27–4.31; MCFD G.3	Before first shift	Annual refresher
First Aid / CPR Level C	OHS Reg. Part 7; MCFD G.3	Within 30 days of hire; before first unsupervised shift	Per certification cycle (annually for CPR/AED; every 3 years for full Level C)
OFA Level 2 (designated first aid attendant)	OHS Reg. Part 7	Before first duty as designated FAA per shift	Per OFA 2 certification cycle



Non-Violent Crisis Intervention (NVCI) — full certification	MCFD Standard G.3 — safe use of restraint; OHS Reg.	Module 1 before solo shift; full certification within 90 days	Annual recertification
Trauma-Informed Practice foundations	MCFD Standard G.3; best practice	Within 60 days	Every 2 years; annual refresher discussion in supervision
Indigenous Cultural Safety and Humility	MCFD Standard G.3; DRIPA	Within 60 days	Annual; San'yas or FNHA approved module
Gender-Affirming Care training (mandatory — Standard G.3.2(b), amended April 2024)	MCFD Standard G.3.2(b) — MANDATORY as of April 2024	Within 60 days	Annual; Trans Care BC or PHSA approved module
LGBTQ2S+ Inclusion and cultural humility	MCFD Standard G.3	Within 60 days	Annual refresher
Suicide and self-harm response (ASIST or SafeTALK)	MCFD Standard G.3; best practice — all direct care staff	Within 60 days	ASIST: biennial; SafeTALK: annual; LivingWorks approved
Medication Administration (authorized staff only)	MCFD Standard G.3; RCR s.46	Before any medication duty	Annual competency check; competency form signed by PC
Working alone safety	OHS Reg. s.4.21–4.24	Before first solo overnight shift	Annual
Driver safety (all who drive youth) + annual driver's abstract	OHS Reg. Part 16; Motor Vehicle Act	Before first driving duty for work	Annual abstract review; driver safety refresher every 3 years

11.2 Additional Training — Program Coordinator Specific

TRAINING	AUTHORITY	TIMELINE	RENEWAL
Supervisory skills and performance management	HR Policy; MCFD G.4	Within 90 days of assuming PC role	Every 3 years or when significant practice updates occur
Progressive discipline and workplace investigations	HR Policy s.13–14	Within 90 days of assuming PC role	Every 3 years
WorkSafeBC incident investigation	OHS Reg. s.3.4; WCA s.172	Before conducting first investigation as PC	Every 3 years; annual update via worksafebc.com
Critical incident reporting and management	MCFD Standards C.2, H.1	Before assuming on-call supervisor responsibility	Annual refresher
ISP coordination and care planning	MCFD Standard E.5	Within 60 days of assuming PC role	At each major MCFD Standards revision



JHSC participation training (if PC serves as JHSC management co-chair)	WCA s.125; OHS Reg.	Within 6 months of joining JHSC	Annual OHS update
Financial management and budget oversight	Finance Manual; CCALA	Within 60 days	Annual finance update
Reflective supervision practice	MCFD Standard G.4; best practice	Within 60 days	Annual; recommend external facilitated training every 2 years
Indigenous Cultural Safety — Advanced Level	DRIPA; MCFD G.3; best practice for supervisors	Within 90 days of assuming PC role	Annual

11.3 Online Training Platforms — Approved Sources

Platform	Training Modules Available
WorkSafeBC — worksafebc.com	WHMIS 2015; Working Alone; Workplace Violence Prevention; Incident Investigation; Young and New Worker; Right to Refuse; OHS Program overview
BC Government eLearning Hub — learning.gov.bc.ca	Rights of Children and Youth in Care (CFCSA s.70); FOIPPA basics; Accessibility and Inclusion; MCFD Standards overview
MCFD — mcf.gov.bc.ca	Mandatory Reporting (CFCSA s.14); Missing Child/Youth Protocol; Child Welfare Policy modules; Critical Incident Reporting
San'yas Indigenous Cultural Safety — sanyas.ca	Indigenous Cultural Safety and Humility Level 1 and 2; specific module for social services and child welfare
FNHA — fnha.ca	Cultural Safety for Health Workers; FNHA Wellness Framework; Indigenous cultural practices
Trans Care BC / PHSA — phsa.ca	Gender-Affirming Care for Youth (MCFD Standard G.3.2(b)); pronoun use; clinical and residential care applications
LivingWorks — livingworks.net	ASIST (2-day, in-person); safeTALK (4-hour); suicide intervention training
Crisis Prevention Institute — crisisprevention.com	NVCI online pre-work and Module 1; verbal de-escalation resources; restraint reduction
BC Campus — bccampus.ca	Trauma-Informed Practice (2–3 hours); open-access social services textbooks
PFLAG Canada — pflagcanada.ca	LGBTQ2S+ inclusion; supporting gender-diverse youth; family conversations
BC OIPC — oipc.bc.ca	FOIPPA for public bodies; privacy best practices for social services
ProQOL — proqol.org	Professional Quality of Life Scale (secondary traumatic stress self-



	assessment); compassion fatigue resources
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PART 12 — DAILY, WEEKLY, MONTHLY, QUARTERLY, AND ANNUAL TASK CALENDARS

12. PC TASK CALENDAR — ALL FREQUENCIES

12.1 Every Shift — Shift Supervisor Responsibilities

☐	Sign in; confirm working-alone device charged and functional (if solo shift)
☐	Receive shift handover from outgoing shift lead: status of every youth; medications; incidents; outstanding tasks; controlled substance count reconciled
☐	Conduct brief safety walkthrough: exits clear; medication cabinet locked; chemical storage locked; all youth located
☐	Review shift log from previous shift for any outstanding items requiring follow-up
☐	Conduct wellness checks per each youth's ISP requirements (minimum every 30 minutes for youth with active safety plans)
☐	Supervise and support direct-care staff through the shift; address any practice concerns immediately
☐	Receive and respond to any hazard reports, behaviour incidents, or youth concerns during the shift
☐	Ensure shift log entries are completed and signed by all staff before departure
☐	Conduct handover to incoming shift lead; confirm all tasks transferred
☐	If on-call: remain available by phone for on-duty staff at all times during the on-call period

12.2 Weekly Tasks

☐	Review shift logs for all youth — minimum 3 logs per youth per week; note any documentation quality issues for supervision feedback
☐	Review school attendance for all enrolled youth; follow up on any absence not documented with an explanation
☐	Review medication records: confirm MAR is current and complete; no unsigned entries; controlled substance count matches log
☐	Check training compliance matrix for any expirations in the next 30 days; arrange coverage or training as needed
☐	Check in with each direct-care staff member (informal, brief): wellbeing check; any concerns from the week
☐	Review emergency on-call coverage for the coming week; confirm schedule is published and all shifts



	covered
☐	Check outstanding corrective actions from facility inspections and CI reviews; follow up on overdue items
☐	Confirm upcoming appointments (medical, dental, school meetings, cultural activities) are scheduled and staff is assigned

12.3 Monthly Tasks — by the 10th of Each Month

☐	Complete Monthly Facility Safety Inspection (Appendix C) — first week of the month; document all deficiencies with priority, owner, and target date
☐	Test all smoke detectors, CO detectors, and emergency lighting; document results on inspection form
☐	Test all fire extinguishers (visual check): confirm charged, tagged, accessible, no damage
☐	Confirm evacuation plan is current and posted on every floor; assembly point is clear
☐	Update Staff Training Compliance Matrix (Appendix E) — by the 5th; identify and address any lapse immediately
☐	Update the OHS bulletin board: WorkSafeBC poster current; JHSC contacts current; emergency contacts current
☐	Review and refill first aid kit(s); document any items used and restocked
☐	Review all Critical Incident Reports from the month; identify patterns; brief Program Manager
☐	Complete Monthly QA Review Tool (Appendix D) and submit to Program Manager — by the 10th
☐	Confirm youth allowances have been paid on schedule; reconcile financial log
☐	Review hot water temperature logs; confirm all readings at or below 49°C (RCR April 2026)
☐	Conduct or confirm scheduled individual supervision sessions for all direct-care staff; complete Appendix G for each
☐	Facilitate team meeting: agenda includes CI review, training update, case consultation, team wellbeing
☐	Review CRC expiry dates for all staff; notify HR of any expiry within 90 days
☐	Submit petty cash reconciliation and expense receipts to Finance per Finance Manual
☐	Confirm Cultural Plans for all Indigenous youth have been actioned in the month (activities, connections facilitated)
☐	Confirm all youth have had school attendance monitored and follow-up completed for any concerns



12.4 Quarterly Tasks

☐	Conduct quarterly fire evacuation drill (announced); document with time, all participants, any deficiencies, corrective actions
☐	Review Critical Incident trend analysis for the quarter; identify any systemic issues for escalation to Program Manager
☐	Confirm all ISPs are on track for quarterly review (meet MCFD 90-day review cycle)
☐	Review emergency on-call staff list; confirm all casuals on the list are still current and available
☐	Submit KPI data to Program Manager for the quarterly leadership review
☐	Conduct quarterly lockdown / shelter-in-place tabletop review at team meeting
☐	Review and update the Environmental Risk Assessment for any youth whose risk profile has changed
☐	Confirm annual fire evacuation drill (unannounced, night or weekend) is scheduled for the current year
☐	Review personnel files: confirm all required documents present and current for each direct-care staff member

12.5 Annual Tasks — July Review Cycle

☐	Participate in Annual MCFD Standards Self-Assessment (Tool A in QA Binder)
☐	Participate in Annual Program Review (MCFD Standard H.2)
☐	Review and update the facility's Environmental Risk Assessment
☐	Conduct annual unannounced fire evacuation drill (night or weekend shift) and document
☐	Conduct annual lockdown / shelter-in-place drill (tabletop acceptable)
☐	Conduct annual workplace violence risk assessment for the facility in consultation with the JHSC
☐	Confirm all vehicle records current: insurance, registration, maintenance, inspection logs
☐	Confirm insurance policies reviewed and adequate (property, vehicle, liability) — report to Program Manager
☐	Review and confirm emergency on-call protocols with all staff at annual team training day
☐	Complete staff driver's abstract review for all staff authorized to transport youth
☐	Complete annual performance reviews for all direct-care staff on the assigned team
☐	Review job descriptions for all direct-care positions; report any required updates to Program Manager
☐	Facilitate Youth and Family Feedback Survey (Tool F); compile results and submit to Program Manager



☐	Review and sign the updated RC Program Manual; ensure all staff sign their acknowledgements within 30 days
☐	Ensure all ProQOL self-assessments are completed by staff (recommend annually)
☐	Review corrective action log: confirm all prior-year actions closed or appropriately escalated



APPENDIX A — PROGRAM COORDINATOR POSITION DESCRIPTION

MCFD Standard G.1 — written job descriptions required for all positions

Position Title	Program Coordinator
Reports To	Program Manager
Supervises	Youth Care Workers; Residential Support Workers; Relief/Casual Staff
Classification / Grade	Grade F — Management (per HR Policy Binder compensation framework)
Employment Type	Full-Time Permanent
Status	Exempt from ESA overtime in certain operational contexts per employment contract

Position Summary

The Program Coordinator is responsible for the day-to-day operational management of one or more Evergreen Community Care residential programs. This includes supervision of direct-care staff; implementation of MCFD Standards A through H; youth safety and rights oversight; training compliance; quality assurance implementation; and on-call supervisory coverage.

Required Qualifications

- Diploma or degree in Child and Youth Care, Social Work, or a recognized related field
- Minimum 3 years of progressive experience in residential child and youth care
- Current Non-Violent Crisis Intervention (NVCI) certification
- Current First Aid / CPR Level C certification
- Current ASIST or equivalent suicide intervention certification
- Demonstrated experience supervising and supporting direct-care staff
- Clear Vulnerable Sector Criminal Record Check
- Valid BC Class 5 driver's licence with satisfactory abstract

Core Competencies

- Trauma-informed leadership: models reflective, strengths-based practice in all interactions with staff and youth
- Cultural safety: actively creates and maintains culturally safe environments for Indigenous and racialized youth and staff
- Rights-based practice: knows and upholding every right in CFCSA s.70 and ensures staff do the same
- OHS management: implements the OHS Program at facility level; responds to hazards and incidents professionally and promptly



- Quality improvement: uses data to drive improvement; tracks corrective actions to completion
- Professional communication: documents accurately; communicates clearly with MCFD, families, and the team
- Resilience and stress management: manages secondary traumatic stress; models self-care and help-seeking



APPENDIX B — NEW STAFF ORIENTATION SIGN-OFF CHECKLIST

Completed by PC on Day 1 of every new hire. All items must be checked before the staff member completes their first independent shift. Both parties sign. File in personnel record immediately.

Staff Name	
Position	
Start Date	
PC / Supervisor	

☐	CRC Vulnerable Sector clearance confirmed in writing by HR before Day 1 of any work with youth
☐	Offer letter and policy acknowledgements signed and filed
☐	Photo ID and access codes issued; IT login set up
☐	Facility tour: all rooms, all exits, fire extinguisher locations, assembly point, first aid kit, AED (if present)
☐	Emergency contacts board shown and explained; staff confirmed they know all numbers
☐	OHS rights explained: right to know, participate, refuse unsafe work — no reprisal
☐	Working-alone check-in device and procedure explained; demonstrated
☐	Medication cabinet location and locking procedure demonstrated
☐	Chemical storage (WHMIS) and SDS binder location shown
☐	PPE location (gloves, masks, eye protection) shown; donning and doffing demonstrated
☐	Evacuation procedure: verbal walkthrough; assembly point practiced
☐	Program Manual Parts 1–5 reviewed with PC: mission, rights, code of ethics, prohibited practices
☐	Rights of Youth in Care (CFCSA s.70): all 14 rights explained; staff demonstrates understanding
☐	Mandatory Reporting (CFCSA s.14): personal, immediate, non-delegable — staff confirms understanding
☐	Prohibited practices list reviewed; staff confirms understanding and acknowledgement
☐	Shift log standards: documentation quality standards explained; sample entry reviewed
☐	ISP and BSP format, location, and reading explained; staff reads BSPs for all current youth
☐	Medication administration process explained (if role includes this); authorization status confirmed



☐	Critical Incident categories and reporting procedure explained
☐	Missing youth protocol steps explained
☐	EFAP access information provided; number given
☐	Mentor assigned; first mentored shift scheduled
☐	Training schedule for first 30, 60, and 90 days provided and reviewed
☐	Youth input on desired staff qualities has been shared with the new staff member
☐	Signed orientation acknowledgement obtained; filed in personnel record today

Staff Signature / Date	
PC / Supervisor Signature / Date	



APPENDIX C — MONTHLY FACILITY SAFETY INSPECTION CHECKLIST

Complete monthly — first week. Document ALL deficiencies with priority level (1=immediate, 2=48 hours, 3=14 days), assigned owner, and target date. Submit to Program Manager within 3 business days.

Facility	
Date of Inspection	
Inspector (PC)	
JHSC Representative Present?	☐ Yes ☐ No

Fire and Life Safety

☐	Smoke detectors — all sleeping areas and common rooms — test one; record result
☐	CO detectors — every level with fuel-burning appliances — test; record result
☐	Fire extinguishers — present, charged, tagged, accessible, pin intact
☐	Emergency lighting — functional at all exits and stairways — test
☐	Exit signs — illuminated and legible at all exits
☐	Evacuation plan — posted on every floor; current and legible
☐	Assembly point — identified and clear of obstruction
☐	Electrical panel — accessible; 1-metre clearance; directory current

Physical Environment

☐	All floors — clean, dry, no trip hazards, no clutter blocking exits
☐	All stairways — clear, handrails secure, adequate lighting
☐	Outdoor areas — no broken equipment, glass, needles, ice, or debris
☐	Hot water temperature at all taps — confirmed not exceeding 49°C — record temperature
☐	All windows — latched appropriately for current youth risk profile
☐	Hazardous items (sharps, knives, tools) — secured per youth safety plans



Health and Safety Equipment

☐	First aid kit — fully stocked; no expired items; location posted
☐	AED (if present) — indicator green; pads not expired
☐	PPE supplies — gloves, masks, eye protection; adequate supply; accessible
☐	Chemical storage — locked; all containers labeled; SDS binder current
☐	Sharps container — properly mounted; not more than 3/4 full

Medication and Records

☐	Medication cabinet — locked; key controlled; contents match MAR
☐	Controlled substance count — current and reconciled per log
☐	Working-alone check-in log — entries complete and current
☐	OHS bulletin board — WorkSafeBC poster current; JHSC contacts current; emergency contacts posted

Fire and Life Safety

☐	Smoke detectors — all sleeping areas and common rooms (test one; record result)
☐	CO detectors — every level with fuel-burning appliances (test; record result)
☐	Fire extinguishers — present, charged, tagged, accessible, pin intact
☐	Emergency lighting — functional at all exits and stairways (test)
☐	Exit signs — illuminated and legible at all exits
☐	Evacuation plan — posted on every floor; current and legible
☐	Assembly point — identified and clear of obstruction
☐	Electrical panel — accessible; 1-metre clearance; directory current

Physical Environment

☐	All floors — clean, dry, no trip hazards, no clutter blocking exits
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☐	All stairways — clear, handrails secure, adequate lighting
☐	Outdoor areas — no broken equipment, glass, needles, ice, or debris
☐	Hot water temperature at all taps — confirmed not exceeding 49°C (record temperature)
☐	All windows — latched appropriately for current youth risk profile
☐	Hazardous items (sharps, knives, tools) — secured per youth safety plans

Health and Safety Equipment

☐	First aid kit — fully stocked; no expired items; location posted
☐	AED (if present) — indicator green; pads not expired
☐	PPE supplies — gloves, masks, eye protection; adequate supply; accessible
☐	Chemical storage — locked; all containers labeled; SDS binder current
☐	Sharps container — properly mounted; not more than 3/4 full

Medication and Records

☐	Medication cabinet — locked; key controlled; contents match MAR
☐	Controlled substance count — current and reconciled per log
☐	Working-alone check-in log — entries complete and current
☐	OHS bulletin board — WorkSafeBC poster current; JHSC contacts current; emergency contacts posted

Deficiency	Priority (1/2/3)	Owner / Target Date



PC Inspector Signature / Date	
Program Manager Review / Date	



APPENDIX D — MONTHLY QA REVIEW TOOL

Complete by the 10th of each month. Submit to Program Manager. This is the Program Coordinator's primary QA accountability document.

Facility / Program	
Month / Year	
Program Coordinator	
Date Submitted	

Quality Indicator	Result / Status this Month	Action Required / Owner / Date
Total Critical Incidents (by category: 1 / 2 / 3)		
All CI reports submitted to MCFD within required timelines? Y/N		
Pattern CI (same youth or same type as prior month)? Y/N		
Physical restraints this month — number; any prohibited technique? Y/N		
All ISPs reviewed on 90-day schedule this month? Y/N		
BSPs updated following any incident? Y/N		
Medication count discrepancies this month — number and resolution		



Medication errors this month — number, type, and resolution		
Facility safety inspection completed? Y/N Deficiencies open:		
Hot water temp checks current? Y/N Any reading above 49°C? Y/N		
Staff training compliance — any lapses? Y/N Gaps identified:		
Individual supervision completed for all staff this month? Y/N		
Team meeting held? Y/N Date:		
School attendance summary — % of scheduled days attended		
Family contact plan followed for all youth? Y/N Issues:		
Community outings completed per ISP? Y/N		
Youth complaints received — number and resolution status		
Cultural Plan activities facilitated for Indigenous youth? Y/N		
Any shifts below minimum staffing		



ratio? Y/N Circumstances:		
Open corrective actions from last month — status of each		
CRC — any staff with expiry within 90 days? Y/N Names:		

PC Summary — Key Findings and Priorities for Next Month:

PC Signature / Date	
Program Manager Review / Date	



APPENDIX E — STAFF TRAINING COMPLIANCE MATRIX

Updated by Program Coordinator by the 5th of each month. Staff with expired training are removed from duties requiring that training immediately. Highlight any expired or expiring within 30 days.

Facility / Program	
Month / Year	
Updated By	

Training (enter expiry date MM/YY)	Staff 1	Staff 2	Staff 3
CRC — Vulnerable Sector (every 5 years)			
Youth Rights (CFCSA s.70) — 2yr			
Mandatory Reporting (s.14) — 2yr			
FOIPPA — annual			
WHMIS 2015 — 3yr			
Abuse Recognition — annual			
Missing Youth Protocol — annual			
Workplace Violence — annual			
First Aid / CPR Level C — annual			
OFA Level 2 (designated FAA) — per cycle			
NVCI — annual recertification			
Trauma-Informed Practice — 2yr			
Indigenous Cultural Safety — annual			
Gender-Affirming Care — annual (MANDATORY G.3.2(b))			
LGBTQ2S+ Inclusion — annual			



ASIST / SafeTALK — biennial / annual			
Medication Admin — annual (if authorized)			
Working Alone — annual			
Driver Abstract — annual			

Training	Staff 4	Staff 5	Staff 6
CRC — Vulnerable Sector			
Youth Rights / Mandatory Reporting			
FOIPPA / WHMIS			
First Aid / CPR Level C			
NVCI — annual			
Indigenous Cultural Safety — annual			
Gender-Affirming Care — annual (MANDATORY)			
ASIST / SafeTALK			
Medication Admin — annual (if authorized)			
Driver Abstract — annual			

PC Signature: _____ Date: _____

**APPENDIX F — CRITICAL INCIDENT SUPERVISOR RESPONSE CHECKLIST**

Complete for every Category 1 or Category 2 Critical Incident. Attach to the CI Report. File in ISR and submit copy to Program Manager.

Youth Name	
Date / Time of Incident	
CI Category	☐ Category 1 ☐ Category 2
PC Notified — Name / Time	
Program Manager Notified — Name / Time	

☐	911 called (if applicable) — Date/Time: _____
☐	Immediate safety ensured: all youth and staff accounted for and safe
☐	First aid provided if needed — First Aid Attendant: _____
☐	On-site staff briefed and roles clarified
☐	MCFD Social Worker notified — Time: _____ OR PCS (1-800-663-9122) called — Time: _____
☐	Program Manager notified — Time: _____
☐	Executive Director notified (if Category 1 involving death, serious injury, or media risk) — Time: _____
☐	Involved youth welfare check completed — Conducted by: _____
☐	Post-incident youth debrief scheduled — Scheduled for: _____
☐	EFAP offered to all involved staff — by end of shift
☐	Involved staff removed from youth-facing duty for remainder of shift if indicated
☐	WorkSafeBC notified by phone if: serious injury, hospitalization, fracture, loss of consciousness — Time: _____
☐	CI Report form completed by shift staff within required timeline
☐	CI Report reviewed and co-signed by PC
☐	JHSC notified within 24 hours if worker safety implications
☐	Post-CI review scheduled within 5 business days — Scheduled for: _____



☐	ISP and BSP review scheduled within 5 business days
☐	Corrective actions identified — entered in corrective action log with owner and target date
☐	WorkSafeBC Form 7 completed within 3 business days (if worker injury requiring medical treatment)
☐	CI Report filed in youth's ISR; copy to Program Manager

PC Signature / Date	
Program Manager Review / Date	



APPENDIX G — INDIVIDUAL SUPERVISION RECORD

Complete for every formal supervision session. File in the staff member's personnel file.
Accessible only to the PC / Program Manager and the staff member.

Staff Name / Position	
Date of Session	
Duration	
Session Number (e.g., Month 3 of 90-day cycle)	
Supervisor (PC)	

Supervision Component	Notes (summarize — not verbatim)
Emotional check-in: how is the staff member doing? Any signs of secondary traumatic stress or compassion fatigue?	
Case review: specific youth discussed; what is going well; what is challenging; what is the staff member doing well	
Practice reflection: one recent situation examined in depth — what happened, what the staff member felt and chose, what they would do differently	
Gender-affirming care practice discussion (MANDATORY per G.4.3(c)) — how is the staff member applying this in daily practice?	
Critical incident or near-miss since last session — was the response appropriate? What support is needed?	



Training status: any overdue training? What professional development goals are active?	
Feedback from supervisor: specific strengths observed; specific growth areas with examples	
Staff member's questions, concerns, or requests	
Action items for the staff member before next session	
Action items for the PC / Program Manager	
Next supervision session scheduled for:	

Staff Signature / Date	
PC Signature / Date	



APPENDIX H — KEY CONTACTS AND REPORTING LINES

Keep this page accessible at all times. Verify currency at monthly inspection.

Contact	Number / Information
Emergency — Police / Fire / Ambulance	911
WorkSafeBC — serious incident (24/7)	1-888-621-7233
MCFD Provincial Centralized Screening (PCS) (24/7)	1-800-663-9122
Advocate for Children and Youth	1-800-476-3933
Helpline for Children	1-800-663-9122
BC Crisis Centre (24/7)	1-800-784-2433
Poison Control	1-800-567-8911
HealthLink BC (nurse line, 24/7)	8-1-1
EFAP (24/7)	[Posted in staff area]
Program Manager	[Name and direct line — posted in staff area]
Executive Director	[Name and line — posted in staff area]
Health Authority Licensing Officer	[Name and number — file with PC]
MCFD Contract Manager	[Name and number — file with PC]
JHSC Worker Co-Chair	[Name and number — posted on OHS bulletin board]
Facility address (for emergency callers)	[Full civic address posted at every telephone]

Version History

Version	Date	Summary
1.0	July 1,	Initial Program Coordinator Manual; all tasks aligned to MCFD SHSS



	2026	Standards A–H, OHS Program (ECC-OHS-POL-2026-002), HR Policy Binder (ECC-HR-POL-2026-001), and QA Program Binder (ECC-QA-2026-001); includes complete training matrix per Standard G.3 including mandatory gender-affirming care training G.3.2(b) April 2024 amendment
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Signatory	Signature / Date
Program Manager	Signature: _____ Date: _____
Executive Director	Signature: _____ Date: _____